Marlowe General Hospital

Department of Internal Medicine - 120 Infirmary Road, Marlowe - Tel 01999 555 0166

Discharge Summary

Patient ID	20-88417	Age and sex	54 years, female
Admitted	3 October 2026 via Emergency	Discharged	9 October 2026 to home
Ward	Medical Ward 4B	Consultant	Dr R. Okonkwo-Hale

Diagnoses

1) 1) Community-acquired pneumonia, right lower lobe.
2) 2) Decompensated heart failure with preserved ejection fraction.
3) 3) Type 2 diabetes mellitus, stable on oral treatment.

Hospital course

The patient presented with five days of cough, fever and breathlessness. Chest imaging showed right lower lobe consolidation and mild pulmonary congestion. She was treated with oral amoxicillin and a short course of intravenous furosemide. Her oxygen requirement settled by the fourth day and she walked unaided on the ward by the sixth.

Vital signs

Measure	On admission
Temperature	38.6 C
Heart rate	104 per minute
Blood pressure	148/88 mmHg
Oxygen saturation	89% on air

Measure	At discharge
Temperature	36.8 C
Heart rate	76 per minute
Blood pressure	128/76 mmHg
Oxygen saturation	96% on air

Discharge medications

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DRUG          DOSE    ROUTE  FREQ  DURATION
Amoxicillin   500 mg  PO     TID   7 days
Furosemide    40 mg   PO     OD    until review
Metformin     850 mg  PO     BD    continue
Paracetamol   1 g     PO     PRN   max 4 g/day
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Laboratory results

Values below were measured on the day of discharge unless a different day is given in the first column.

Test	Result	Unit	Reference range	Flag
Haematology				
Haemoglobin	118	g/L	120 to 160	Low
White cell count	9.4	x10^9/L	4.0 to 11.0	
Platelets	312	x10^9/L	150 to 400	
C-reactive protein, day 1	186	mg/L	under 5	High
C-reactive protein, day 6	22	mg/L	under 5	High
Biochemistry				
Sodium	139	mmol/L	135 to 145	
Potassium	4.1	mmol/L	3.5 to 5.0	

Test	Result	Unit	Reference range	Flag
Creatinine	86	umol/L	45 to 90	
Estimated GFR	68	mL/min	over 60	
HbA1c	49	mmol/mol	under 48	High
NT-proBNP, admission	2140	ng/L	under 300	High
NT-proBNP, discharge	790	ng/L	under 300	High
Microbiology				
Blood cultures	No growth at 5 days			
Sputum culture	Mixed oral flora			

Follow-up

- - Heart failure clinic in two weeks, with repeat blood tests the day before.
- - Chest radiograph in six weeks to confirm resolution of the consolidation.
- - Practice nurse to review the diabetes plan within one month.

Discharge checklist

- ☒ Medication reviewed with the patient
- ☒ Written information on heart failure given
- ☐ District nurse visit arranged
- ☒ Transport home arranged

Advice to the patient

Please weigh yourself every morning and call the ward if your weight rises by more than two kilograms in three days. Return to the Emergency Department at once if you become short of breath at rest.